

Basic Principles of Medicine II Behavioral Sciences

Sociocultural Sensitivity and Awareness in Healthcare

Introduction

In this session, students will examine how communication style, language barriers, health beliefs, family roles, social context, clinician assumptions, and structural barriers can affect patient-physician interactions and health outcomes. Through five brief video cases adapted from the "Cultural Awareness and Sensitivity" training series by the American College of Obstetricians and Gynecologists, students will identify factors that influence trust, disclosure, adherence, and shared decision-making in clinical care.

Duration of session: ~70 minutes (20 minutes for case study videos + 50 minutes for discussion)

Objective: SOM.PB.VI.BPM2.5.NB.5.BEHS.0162 Discuss how social and cross-cultural factors, such as language, decision-making styles, family dynamics, beliefs, and practices, affect patient-physician interactions and healthcare outcomes, highlighting the essential role of cultural competence in providing effective patient-centered care

Instructions for Students

1. **Pre-assessment:** Complete the self-assessment before the small group session.
[Link to self-assessment website](#)
2. While viewing each case, make notes on the following:
 - a. **Communication behaviors:** What verbal and non-verbal communication skills did the clinician use?
 - b. **Patient cues:** What verbal or non-verbal signals suggested discomfort, confusion, mistrust, embarrassment, or hesitation?
 - c. **Contextual factors:** What sociocultural, linguistic, family, or structural factors may be influencing the interaction?
 - d. **Clinical response:** What could the clinician or healthcare team have done differently to improve trust, understanding, and care?
3. Be prepared to discuss how these factors affect diagnosis, disclosure, adherence, follow-up, and patient outcomes.
4. **Post-assessment:** Complete the self-assessment after the small group session.
[Link to self-assessment website](#)

Note: These cases are designed to promote reflection, not stereotyping. Avoid assuming that all patients from a particular group share the same beliefs or

preferences. Focus on the individual patient's perspective, the clinician's communication, and the healthcare context.

Case Study 1: Ms. Wong

Video 1a (5 minutes)

1. What communication behaviors did the physician use effectively during this encounter?
2. What communication behaviors could have been improved?
3. What verbal or non-verbal cues suggested that the patient had concerns about the blood test?
4. What assumption did the physician appear to make about the patient's reluctance, and why is that important?
5. What should the physician have asked next to better understand the patient's concern?

Video 1b (5 minutes)

1. What did the physician do differently in this second interaction to better explore the patient's concern?
2. What belief or concern influenced the patient's decision not to have her blood drawn?
3. How might similar beliefs or concerns affect decision about other medical procedures, such as biopsy, transfusion, surgery, transplant, or autopsy?
4. What are three specific strategies clinicians can use to explore patient beliefs without making assumptions?
5. How could failure to address this concern affect adherence, follow-up, and health outcomes?

Case Study 2: Alya Ahmad

Video 2a (5 minutes)

1. What communication behaviors did the physician use effectively at the start of the encounter?
2. At what point did the interaction begin to break down, and why?
3. What verbal or nonverbal cues suggested that the patient was becoming uncomfortable?
4. What factors may have contributed to the patient's reluctance to answer the physician's questions?
5. What should the physician have said or done differently before continuing with more sensitive questions or examination?

Video 2b (5 minutes)

1. What did the physician do differently in this second interaction to better understand and respond to the patient's discomfort?
2. Why was it important to address both the physician's gender and the interpreter's role in this encounter?
3. Which patients may require interpreter services or other communication accommodations in clinical care?
4. What are the benefits and limitations of interpreter or translation services in healthcare?
5. Why are ad hoc interpreters, including family members or children, often problematic in medical encounters?
6. How could failure to address the patient's discomfort affect disclosure, trust, examination, and follow-up care?

Case Study 3: Sara

Video 3a (5 minutes)

1. What communication behaviors did the physician use effectively at the start of the encounter?
2. What communication behaviors could have been improved during the sexual and reproductive history?
3. What verbal or nonverbal cues suggested that the patient was becoming uncomfortable or defensive?
4. What assumptions did the physician appear to make, and how might those assumptions affect trust and disclosure?
5. What should the physician have said or asked differently to obtain the needed history without making assumptions?

Video 3b (5 minutes)

1. What did the physician do differently in this second interaction to make the discussion more respectful and patient-centered?
2. Which questions or phrases helped reduce assumptions and improve understanding?
3. What communication strategies can clinicians use when asking about sexual history, partners, pregnancy intentions, or reproductive risk?
4. Why is it important to explain why sensitive questions are being asked?
5. How can assumptions about sexual behavior, partner gender, or future pregnancy goals interfere with care?

Case Study 4: Jeanine

Video 4a (5 minutes)

1. What communication behaviors did the front-desk staff member use during this interaction?
2. Which behaviors were helpful, and which may have increased the patient's distress or shame?
3. What verbal or nonverbal cues suggested that the patient was overwhelmed, embarrassed, or considering leaving?
4. What assumptions might staff make about a patient who arrives late with children, and why can those assumptions be harmful?
5. What factors may have contributed to the patient's early departure from the clinic?

Video 4b (5 minutes)

1. What did the front-desk staff member do differently in this second interaction to support the patient?
2. How did privacy, empathy, and practical problem-solving change the interaction?
3. What are some social or structural barriers to healthcare access, and which ones were applicable in this case?
4. Why is the front desk part of the patient's therapeutic experience, even though the interaction is not with the physician?
5. What could the clinic or healthcare team do at a systems level to better support patients facing similar barriers?

Case Study 5: Mrs. Hernandez

Video 5a (5 minutes)

1. What communication behaviors did the healthcare provider use during this discussion?
2. Which behaviors were helpful, and which may have sounded judgmental or blame-focused?
3. What verbal or nonverbal cues suggested that the patient felt hesitant, embarrassed, or judged?
4. What are some possible reasons a patient may not follow a recommended treatment plan?
5. What should the clinician have asked next to better understand why the patient was not using the medication?

Video 5b (5 minutes)

1. What did the healthcare provider do differently in this second interaction to better understand the patient's perspective?

2. How did the clinician address the patient's use of traditional or herbal remedies in a more respectful and patient-centered way?
3. Why might patients hesitate to disclose traditional, complementary, or home remedies to clinicians?
4. How can clinicians ask about these practices in a way that supports trust while also protecting safety?
5. What are the potential benefits and risks of using traditional, complementary, or integrative practices alongside conventional treatment?
6. How could collaboration with the patient and family improve adherence, safety, and follow-up?

Test Your Understanding (Answer key with explanations are on the last page)

1. A 29-year-old woman comes to the physician because of a 2-day history of severe stomach pain and blood in her urine. She speaks fluent Spanish with limited English proficiency and is accompanied by her bilingual husband. The physician noticed several bruises on her arms and neck during the examination. No professional Spanish interpreters are available at the hospital that day. Which of the following is the most appropriate method for the physician to facilitate communication during this clinical encounter?

- A. Interview her alone in simple English
- B. Interview her with her spouse as an interpreter
- C. Reschedule the appointment for another day when the Spanish interpreter is available
- D. Utilize a phone interpreter service, which is immediately available

2. A 2-year-old boy is brought to the emergency department by his parents because of a 3-hour history of recurrent generalized tonic-clonic seizures. He was prescribed anti-epileptic medication during a previous emergency department visit for seizures. However, his parents have not administered the medication, nor have they followed up with the recommended pediatric neurologist. Which of the following is the most appropriate initial step for the physician to improve treatment adherence?

- A. Inform the family about the risks of seizures
- B. Inform the family about the anti-epileptic benefits of the medication
- C. Inform the family about the medication side effects
- D. Ask the family to explain their understanding of the reason their son is having seizures
- E. Inform the family about the etiology of their son's epilepsy syndrome

Answers with Explanations

1. Answer: D

A phone interpreter ensures accurate communication while maintaining confidentiality and patient safety. It is the best available option when no on-site interpreter is present.

It is advisable to communicate with this patient without her spouse present, as the bruises on her arms and neck suggest the possibility of abuse that she may not disclose in her husband's presence. This situation would be further complicated if her husband were to serve as the interpreter. Women with limited communication skills are particularly vulnerable to abuse. Thus, physicians must be cautious not to inadvertently enable this situation in their eagerness to provide prompt and efficient treatment. In this case, a phone interpretation service would be the best option to ensure efficient, effective, and culturally sensitive communication between the physician and the patient.

Option A: the patient has limited English proficiency, and critical details may be missed or misunderstood. She may lack the vocabulary to express her pain or its contributing factors effectively. This approach compromises accurate communication and patient safety.

Option B: it is inappropriate to use family members as interpreters, especially in cases where abuse may be suspected. Family members may misinterpret or withhold information, which can endanger the patient. Option C: rescheduling the appointment is also not advisable, as her symptoms indicate the need for immediate intervention.

2. Answer: D

The most appropriate first step in improving treatment adherence is to assess the family's understanding of the child's condition and the rationale for the prescribed treatment. Nonadherence to medication can stem from various factors, including a lack of understanding, cultural beliefs, fear of side effects, or mistrust of the healthcare system. By asking the family to explain their understanding, the physician can identify misconceptions, knowledge gaps, or specific concerns that need to be addressed. This approach fosters effective communication and shared decision-making, which is essential for improving adherence.

Informing the family about the risks of seizures (A), the anti-epileptic benefits of the medication (B), the medication side effects (C), and the disease etiology (E) are all important aspects of physician-patient communication; however, these actions would not help the physician understand why the family did not comply with treatment recommendations in the first place and would not necessarily improve treatment compliance. The most important action the physician could take would be to try to understand the barriers to treatment compliance and address them directly. The family may have an alternative explanatory model of illness, which leads them to question the necessity of medication and may even lead them to distrust the medication. However, the physician will not understand any of this without asking.